

Chronic Telogen Effluvium

A persistent form of telogen effluvium, characterized by loss of hair volume lasting more than 6 months, typically occurs in middle-aged women who may bring large amounts of shed hair for examination at a clinic visit. Examination reveals normal or high hair density, although some rarefaction with short hairs may be observed along the frontotemporal regions.

A simple method for evaluating shed hairs—based on their length and the number of hairs shed per day—has been proposed to help distinguish chronic telogen effluvium from androgenetic alopecia. In androgenetic alopecia, shed hairs are typically shorter than 3 cm and fewer in number per day, whereas in chronic telogen effluvium, longer hairs are shed in greater quantities.

Underlying medical conditions such as thyroid deficiency and systemic lupus erythematosus (SLE) should be ruled out. Iron deficiency, with or without anemia, is frequently cited as a potential cause of chronic telogen effluvium, though this association remains controversial. One study found that mean ferritin levels were significantly lower in patients with androgenetic alopecia compared to controls without hair loss, but no such difference was observed in patients with telogen effluvium.

In some women, chronic telogen effluvium may represent an early manifestation of androgenetic alopecia. Up to 60 percent of women with chronic telogen effluvium may show histologic features of pattern hair loss on biopsy. However, in many cases, no definitive cause is identified, and the persistent shedding may reflect an age-related shortening of the hair cycle. This process can be difficult to differentiate from androgen-dependent anagen shortening, suggesting a possible overlap or relationship between the two mechanisms.

Retinoid-induced telogen effluvium—particularly from isotretinoin used in the treatment of acne—has been reported as a trigger for the onset of androgenetic alopecia in older teenagers and young adults.

Diffuse, non-scarring hair loss with excessive hair shaft shedding is also a common feature of systemic lupus erythematosus (SLE), occurring in 24 to 84 percent of patients. The severity of telogen effluvium often correlates with disease activity. Some SLE patients may later develop cicatricial alopecia due to chronic discoid lupus lesions.

Anagen Effluvium

Shedding of telogen hairs is a normal daily occurrence, but shedding of anagen hairs is always abnormal. The term *anagen effluvium* is somewhat misleading because the affected anagen hairs are typically not shed intact but rather break off due to damage.

In contrast, in conditions such as loose anagen syndrome and alopecia areata, anagen hairs are usually shed completely (in toto).